

# Patient C Clinic Packet

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Privacy posture: de-identified clinic-support packet. No legal name, date of birth, address, phone, employer name, child name, or direct identifiers included.

## PURPOSE

This packet summarizes Patient C's reported history and current concerns so they do not have to explain everything from scratch at each appointment.

This is not a diagnosis. It is a structured medical-history and appointment-prep note for review by licensed clinicians.

## PATIENT SNAPSHOT

- Adult patient, de-identified as Patient C.
- Height: 5 ft 5 in.
- Weight: approximately 270 lb.
- Current life stress: only household income, not making enough to cover bills, high financial pressure.
- Work strain: physically and emotionally demanding daycare teaching job.
- Emotional state: depressed, anxious, trauma burden, decision paralysis, and not wanting to feel alone.

## MAJOR REPORTED HISTORY

### Reproductive / Pregnancy / Postpartum

- Birth control use for 10+ years.
- History of pregnancy and delivery with epidural.
- C-section history.
- Infection in C-section scar area after birth.
- High blood pressure during pregnancy.
- Diagnosed with preeclampsia after birth.
- Current heavy bleeding and cramping with periods.
- Nausea during periods.

### Sleep

- Lifelong trouble sleeping.
- When sleep happens, can sleep for 10+ hours.
- Sleep quality, fatigue, long sleep duration, depression/anxiety, chronic pain, heavy bleeding, thyroid/metabolic causes, and possible sleep disorder should be reviewed.

### Dental / Nutrition

- Dental issues for approximately 8 years.
- No natural teeth currently reported.
- Uses dentures.
- Denture fit, nutrition, chewing limits, and oral-health follow-up may affect energy, diet, and mood.

### Neurologic

- At least 2 prior concussions.
- Current review should ask about headaches, dizziness, memory/focus, irritability, sleep disruption, visual symptoms, and worsening mood after head injuries.

### Gastrointestinal / Surgical

- History of gallstones.
- Gallbladder removed.

### Musculoskeletal / Pain

- Ribs reportedly twist/out of place.
- Sees chiropractor for body adjustments.
- Shoulder pain.
- Back pain.
- Feet pain.
- Chronic pain may be contributing to poor sleep, anxiety/depression, fatigue, and work stress.

### Mental Health / Social Stress

- Trauma burden.
- Depression.
- Anxiety is high.
- Decision paralysis.
- Extreme financial stress.
- Feels alone / does not want to feel alone.
- High-demand caregiving/teaching job while being the only income source.

### CURRENT TOP CONCERNS TO BRING TO A CLINICIAN

- <b>1.</b> Heavy bleeding, cramping, and nausea during periods.
- <b>2.</b> Sleep difficulty with long sleep episodes when able.
- <b>3.</b> Depression, anxiety, trauma, decision paralysis, and isolation.
- <b>4.</b> Chronic rib/back/shoulder/feet pain.
- <b>5.</b> Past high blood pressure during pregnancy and postpartum preeclampsia.
- <b>6.</b> Weight-related health risks, without blaming or reducing the patient to weight.
- <b>7.</b> Prior concussions and possible long-term symptoms.
- <b>8.</b> Denture/dental history and nutrition limitations.

### SUGGESTED PRIMARY-CARE APPOINTMENT REQUESTS

Ask primary care to consider:

- Blood pressure check and home blood-pressure log plan.
- CBC.
- Ferritin and iron studies.
- TSH / thyroid screen.
- CMP / kidney and liver function.
- A1c / diabetes screen.
- Lipid panel.
- Urine protein/albumin or kidney-risk check if appropriate given preeclampsia history.
- Medication and birth-control review.
- Sleep-apnea screening and possible sleep-study referral.
- Mental-health treatment plan, therapy referral, and medication discussion if Patient C is open to it.
- Physical therapy referral for chronic rib/back/shoulder/feet pain.
- Dental/denture follow-up if fit, chewing, pain, or nutrition are issues.

### SUGGESTED OB/GYN APPOINTMENT REQUESTS

Ask OB/GYN to evaluate:

- Heavy menstrual bleeding.
- Severe cramps.
- Nausea during periods.

- Whether current birth control is helping, worsening, or masking symptoms.
- Possible anemia/iron deficiency from bleeding.
- Possible fibroids, endometriosis, adenomyosis, ovarian cysts, hormonal issues, or other causes of abnormal uterine bleeding.
- Whether imaging, labs, or treatment changes are needed.

#### SUGGESTED MENTAL-HEALTH SUPPORT REQUESTS

Ask for help with:

- Depression and anxiety treatment plan.
- Trauma-informed therapy options.
- Decision paralysis / executive-function overload.
- Financial stress and caregiver/teacher burnout.
- Sleep routine support.
- Crisis plan if Patient C feels unsafe, trapped, or at risk of self-harm.

#### RED FLAGS / URGENT CARE TRIGGERS

Seek urgent medical help or emergency care if any of these occur:

- Chest pain.
- Severe shortness of breath.
- Fainting.
- Stroke-like symptoms: face droop, arm weakness, speech trouble, sudden confusion, sudden severe dizziness, or sudden vision changes.
- New severe headache, especially with vision changes or neurological symptoms.
- Blood pressure around 180/120 or higher, especially with symptoms.
- Heavy bleeding soaking through a pad/tampon every hour for several hours.
- Large clots, severe weakness, dizziness, or feeling like they may pass out during bleeding.
- Severe abdominal/pelvic pain.
- Thoughts of self-harm, not being safe, or feeling unable to stay safe.

#### ONE-PARAGRAPH DOCTOR SUMMARY

Patient C is an adult patient, 5 ft 5 in and approximately 270 lb, with 10+ years of birth-control use, severe dental history resulting in dentures, pregnancy history with epidural and C-section, C-section scar infection, high blood pressure during pregnancy, postpartum preeclampsia, at least two concussions, gallstones with gallbladder removal, chronic rib/back/shoulder/feet pain treated with chiropractic adjustments, heavy painful periods with nausea, lifelong sleep difficulty with 10+ hour sleep episodes when sleep occurs, depression/anxiety/trauma, decision paralysis, high financial stress, and a physically/emotionally taxing daycare teaching job. Main current concerns are heavy bleeding/cramping/nausea, chronic sleep problems, chronic pain, mental-health strain, cardiovascular risk after pregnancy hypertension/preeclampsia, and overall exhaustion.

#### CARE TEAM PRIORITY ORDER

- <b>1.</b> Primary care: whole-picture workup, blood pressure, labs, sleep, referrals.
- <b>2.</b> OB/GYN: heavy bleeding/cramping/nausea and birth-control review.
- <b>3.</b> Mental health: depression, anxiety, trauma, decision paralysis, isolation.
- <b>4.</b> Physical therapy / musculoskeletal evaluation: pain pattern and function.
- <b>5.</b> Dental/denture follow-up: fit, chewing, nutrition, oral pain.

#### NOTES FOR PATIENT C

Patient C should not have to carry all of this alone or explain it perfectly under stress. Bringing this packet to a primary-care visit can help the appointment start faster and keep the focus on relief, safety, and next steps.